



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Procedure

Forced Entry and Security of a Mental Health Consumer's Residence

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health

1. Introduction

The purpose of this procedure is to provide guidance to mental health service staff regarding securing and repairing a consumer's residence in the event police have been required to force entry to enable an emergency mental health assessment of the consumer or to establish their physical wellbeing.

2. Terminology

Forced entry	Entry into a building by force e.g. by forcing a lock.
Authorised Mental Health Practitioner (AMHP)	A mental health practitioner designated by the Chief Psychiatrist as having the qualifications, training and experience appropriate for performing the functions of an Authorised Mental Health Practitioner under the Mental Health Act 2014.
Community visit	Provision of clinical care by a staff member in a community setting, outside the physical location of the hospital.
At risk community visit	A community visit with pre-identified or anticipated risk factors which constitute a potential threat to the safety of staff/visitors.

3. Procedure

3.1. Initial consideration

- Where mental health clinicians are concerned or a third party raises significant concerns about the mental state or physical welfare of a mental health consumer in the community, a mental health clinician will endeavour to undertake a mental health assessment of the consumer.

3.2. Prior to home visit

- Where possible clinicians will seek collateral information prior to assessment.
- Mental health clinicians will attempt to contact:
 - the patient
 - family
 - next of kin.
- If contact cannot be made the clinician in discussion with Multidisciplinary Team (MDT) members and Consultant Psychiatrist will instigate a home visit.
- If clinicians have been made aware that they will be unable to access the residence, they will contact family or other potential key holders to organise access to the residence.
- Two staff members must attend at risk community visits. One staff member must be either a Medical Practitioner or an AMHP whenever possible.
- Where possible Fremantle Hospital Engineering Department will be provided with advanced notice of the potential forced entry to a private residence (FSH does not require advance notice).

3.3. At location

- If on arrival, the consumer is not allowing entry, has barricaded themselves or is not responding and the clinician is unable to conduct a face-to-face assessment and has ongoing concerns for the persons safety, in discussion with MDT members and Consultant Psychiatrist they may request WA Police Force assistance.
- WA Police Force assistance may be requested if mental health clinicians have serious and imminent concerns for the safety of the consumer or the risks posed to the clinician at a community visit.
- Should WA Police Force staff decide not to attend, the situation can be escalated to the Shift Superintendent who will make the final decision.
- Where possible a photo will be taken of the residence entry point pre and post access to the residence.
- WA Police will if required, force entry via the most practical entry point to limit property damage.

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- On completion of the assessment/home visit the clinical staff will request family/next of kin to remain at the premises until repairs are made and it is secure.
- Where there is no family/next of kin and the consumers residence is not adequately secure the staff member will remain on site or arrange another staff member to remain on site until the arrival of the repairer, if safe to do so.
- The senior clinician at the premises will inform the following of the situation, location of the premises and the nature of repairs required e.g. carpenter locksmith or glazier.
 - FH Business hours (08.30 – 16.30) - Engineering Department
 - FH After hours (16.30 – 22.00) - Shift Engineer via Fremantle Hospital switch
 - FSH Business hours (08.30 – 16.30) – Mental Health Nurse Director

3.4. Post event

- Clinical staff must document events in the consumer's medical record including:
 - reasons WA Police Force were required to force entry to the residence.
 - any damage to the property
 - identity of the attending police officers
 - actions taken to secure the property.
- Inform their line manager of the forced entry and actions taken to secure the property.
- Raise an EMPAC for repairs required.
- Report any clinical incidents through the Datix Clinical Incident Management System (CIMS)
- FH Engineering services will:
 - Arrange the appropriate contractor to undertake the work.
 - Raise a purchase order at the first available opportunity.
 - Allocate all related charges to the MHS cost centre.

4. Compliance/performance monitoring

Any clinical incident related to the forced entry and security of a mental health consumers residence will be documented using Datix Clinical Incident Management System and monitored by the Service 5 Safety Quality Risk Committee.

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5. Related policy documents

DoH:

- [Safety Planning for mental health consumers](#)
- [Community Mental Health Status Assessments Role of Mental Health Clinicians](#)
- [Principles and best practice for the care of people who may be suicidal](#)
- [Principles and best practice for the care of people who may be at risk of exhibiting violent or aggressive behaviour](#)
- [Clinical Incident Management](#)

SMHS:

- [Acute response mental Health Emergencies/Crises](#)
- [Community/Home visiting](#)

FSFHG

- Clinical risk: Assessment and management (FSFH-HW-POL-0017)
- Clinical care of people who may be suicidal (FSFH-HW-POL-0037)
- Clinical Documentation (FSFH-HW-POL-0039)
- Community Mental Health Status assessment (FSFH-MENH-PRO-0006)

6. Related documents

[The Mental Health Act 2014](#)

7. Related standards

NSQHS Standards:

- Clinical Governance
- Recognising and responding to acute deterioration

The Chief Psychiatrists Standards for Clinical Care

- Assessment
- Risk assessment and Management

8. Authorisation

EXECUTIVE SPONSOR: Nurse Director – Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	11/2016	CNS, Clinical Improvement	FHMHS Clinical Governance Committee	Policy or Executive Committee	11/2019
2	10/2020	CNS, Clinical Improvement	Service 5 Safety Quality Risk Committee	FSFHG Policy Committee	10/2024
3	11/2024	CNS, Clinical Improvement		Service 5 Safety Quality Risk Committee	12/2028
4	03/2026	FSFHG Policy Team Minor template update only		FSFHG Policy Committee	12/2028

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